



INDIAN SOCIETY

MODULE - 13

INDIAN SOCIETY - HEALTH

INDIAN SOCIETY

HEALTH SCENARIO

1 In Global Burden of Disease rankings for healthcare access and quality, India is at 145/195 countries. India's neonatal mortality ranking is worse than Afghanistan and Somalia,

2 India is committed to SDG 3 i.e., health and well-being to all by 2030

3 **Mental health:** As per WHO, 8 lakh people die by suicide globally and over 1.3 lakh in India. Among 15-29 year, it is the 2nd leading cause of death.

4 India is home to more than one-third of the world's stunted children (Global Nutrition Report).



INDIAN SOCIETY

HEALTH SCENARIO

As per NSO survey 2019,

- a** Around 14% of rural population and 19% of urban population had their health expenditure covered. Only 1% of urban population received health insurance from government health protection schemes.
- b** The average medical expenditure that is being spent in rural India per hospitalization case is Rs 16,676 and in urban India is Rs 26,475.
- c** The survey says that about 90% of rural childbirths and 96% of urban child births were institutional.
- d** In rural areas, around 97% of the children received vaccination. This includes all the 8 prescribed vaccinations.
- e** Among them, 59% of boys and 60% of girls were completely immunized.



After the implementation of economic planning a lot of progress has been made in health services in India after Independence.

- 1 Basic infrastructure in the form of primary health care services, has been provided in urban and rural areas.
- 2 Primary health care services include maternal and child health care services and family welfare services.
- 3 Specialised health care or Tertiary health services are provided through hospitals in urban areas.

- 4 Mukherjee Committee in 1966 prescribed a system of targets and incentives and identified ANMs as village-level workers and agents for the popularization of the health programmes.

ANMOL (ANM ON LINE)
Tablet based application for ANMs



- 5 In 1975, the Srivastava Committee recommended expansion in the role of ANM. Recommended expansion included the role of an ANM as a multipurpose health worker.
- 6 In 2005, the National Rural Health Mission (NRHM) was launched, which focused on improvising primary health care in villages and further increased the importance of the ANM as a link between health services and the community.
- 7 ANMs work at health sub-centres. The sub-centre is a small village-level institution that provides primary health care to the community. The sub-centre works under the Primary Health Centre (PHC).
- 8 Each PHC usually has around six such sub-centres. In 2005 NRHM made provision of two ANMs (one permanent and one contractual) for each sub-centre. The ANM is usually selected from the local village to increase accountability.



HEALTH CARE FRAMEWORK IN INDIA

9 ANM works include

- a** Maternal and child health along with family planning services
- b** Health and nutrition education
- c** Efforts for maintaining environmental sanitation
- d** Immunisation for the control of communicable diseases
- e** Treatment of minor injuries, and first aid in emergencies and disasters.

- f** In remote areas, such as hilly and tribal areas where transport facility is likely to be poor, ANMs are required to conduct home deliveries for women.



HEALTH CARE FRAMEWORK IN INDIA

10 Relationship with ASHA

- a** The Accredited Social Health Activist (ASHA) is a community health worker. Depending on the area covered by the sub-centre, each ANM is supported by four or five ASHAs.
- b** ANMs are supposed to take weekly or fortnightly meeting with ASHAs to review work done the last week or fortnight. ANMs guides ASHAs on aspects of health care.
- c** With the Anganwadi Worker (AWW), the ANM acts as a resource person for the training of ASHAs.
- d** Anganwadi is a type of rural child care centre in India. They were started by the Indian government in 1975 as part of the Integrated Child Development Services program to combat child hunger and malnutrition.
- e** The ASHA brings pregnant women to the ANM for check-ups, married couples to the ANM for counselling on the family planning, children to immunisation sessions held by the ANM. In a way the ASHA act as bridge between the ANM and the village.



HEALTH CARE FRAMEWORK IN INDIA

PROBLEMS

1 A weak primary health care sector with disparities across country. Ex. in 2011, 6/10 hospitals in the less developed states did not provide intensive care and struggle with issues like sanitation and drainage.

2 Urban areas have 73% of the public hospital beds, even when 69% population is rural.

3 Unequally distributed skilled human resources: only 10% of Indian doctors work in public health sector

4 Only 1 doctor per 1700 people against WHO norms of 1:1000. 2015 Parliamentary Report said that even if we add 100 colleges per year, it will take up to 2029 to meet WHO norms.

The Countries With The Highest Density Of Doctors

Number of medical doctors per 1,000 inhabitants in selected countries*

* 2018 or most recent year data available
Source: OECD

statista

HEALTH CARE FRAMEWORK IN INDIA

PROBLEMS

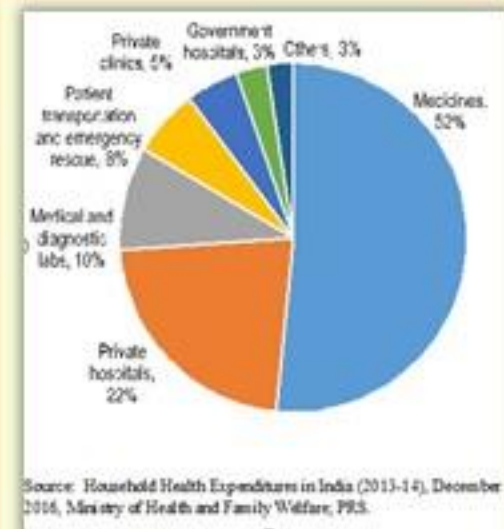
- 5 **Large unregulated private sector** over 60% and provider induced demand (unnecessary tests, never prescribe generic drugs, info system is scattered - no proper database => repeat all tests, delay, OOP)
- 6 **Low public spending** on health at 1.15% of GDP while China spends 3% of GDP. India's spending is falling as it was 1.47% in 1986.
- 7 **Fragmented health information systems:** Although India enacted a law on the mandatory registration of births and deaths in 1969, only 86% of births and 71% of deaths were registered in 2013.
- 8 **Poor Health Cover:** Over 80% of India's population is not covered under any health insurance scheme. Also, RSBY could only cover 12% of urban and 13% of rural population.



HEALTH CARE FRAMEWORK IN INDIA

PROBLEMS

- 9 High out of pocket expenditure:** most of it is spent on medicines. India spends only a little more than 0.1% of GDP on publicly funded drugs. As per WHO , 63% OOP is responsible for 1/6th of India's poverty!
- 10 WORLD BANK** - India loses 6% of GDP due to preventive illness and premature deaths.
- 11** Lack of trained health work force: For a population of 1.02 billion in 2001, there were just 20 lakh health workers in India.
- 12 Lifestyle diseases:**
- India is the third most obese country in the world
 - By 2030, India is expected to be the diabetes capital of the world. (Lancet)
 - Four NCDs - cardiovascular diseases, cancer, chronic respiratory diseases and diabetes make most contribution.
 - WEF estimated that India stands to lose \$4.58 trillion before 2030 due to NCDs and mental health conditions.



HEALTH CARE FRAMEWORK IN INDIA

PROBLEMS

13 Twin Burden of Over consumption and Under Nutrition: NFHS 2015-16, 38% of children under the age of five are stunted and 21% are wasted. 9% overweight and 3% obese.

14 **Nutrition issues:** As per Global Nutrition Report, Stunting - 114/132 (39% incidence), Wasting - 120/130 (15%), Anaemia in women - 170/185 (48%)

15 **Corruption,** no public health cadre

16 Morbidity pattern shifting from communicable to non-communicable, but health facilities didn't adapt

17 There is a need for promotive, preventive, curative and rehabilitative health system but our system is only curative.



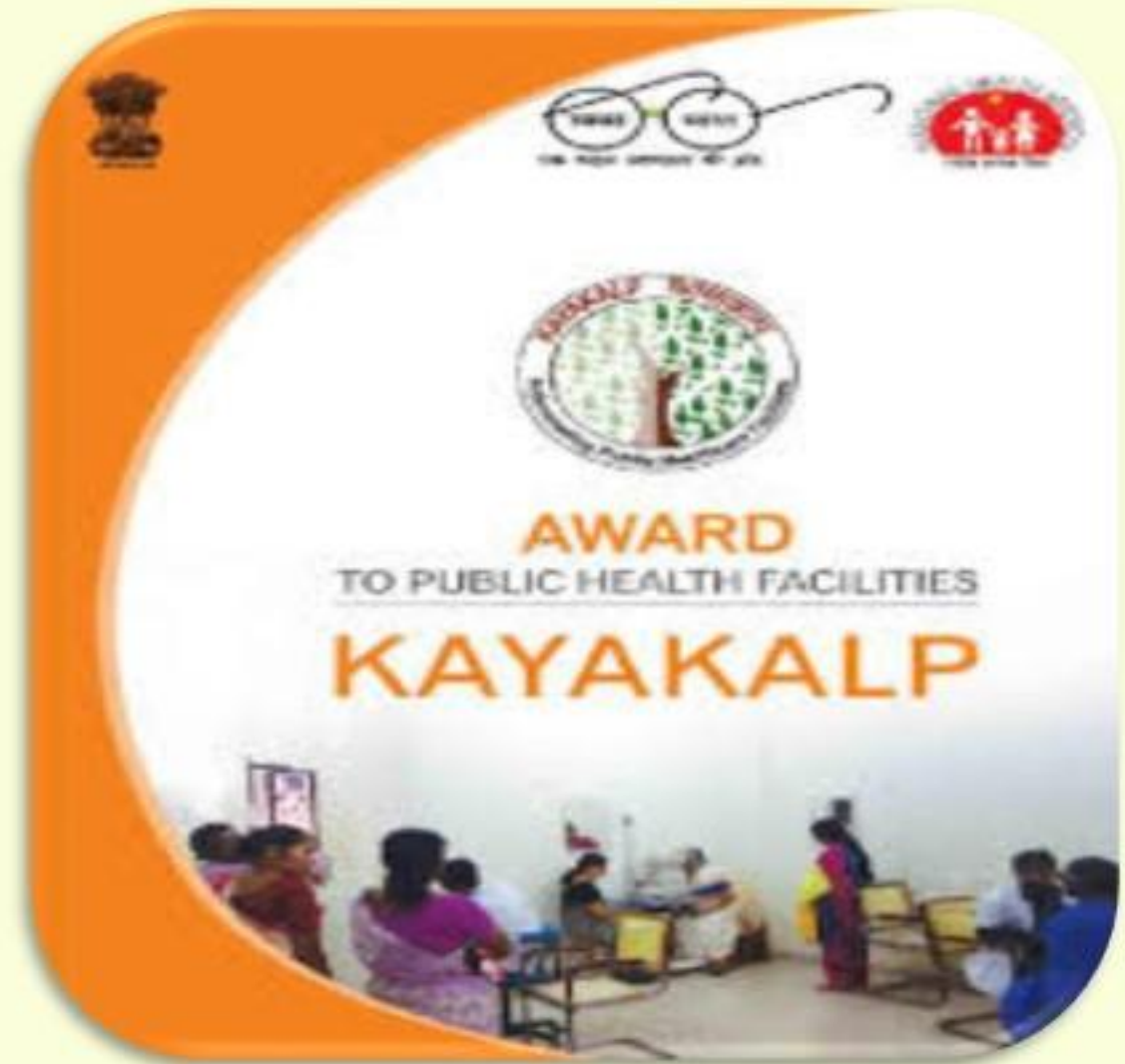
1 Adequate investment of public financial resources in health: In numerous countries, public and private providers compete with each other – Canada and Taiwan.

2 Preventive health care rather than curative care: support evidence-based preventive health interventions like breastfeeding, tackle risk factors from smoking, high blood pressure and sanitation. Make use of traditional medicine and yoga in schools.

3 Challenges in human resources: strengthen grassroots work by ASHAs. Ex. China had three million bare-foot doctors during the seventies to carry out these functions.



- 4** Convergence with programs addressing the key social determinants of health (nutrition, drinking water and sanitation)
- 5** Use Kayakalp awards, Electronic Health Records, AYUSH
- 6** 9 sanctioned AIIMS should be made fully functional
- 7** ASHAs and Anganwadi Workers (AWWs) should be offered performance-based admissions to Auxiliary Nurse Midwife (ANM) schools for career progression



1 Increase spending to 2.5% of GDP by 2025 from current 1.15%. States should spend 8% of their budget towards health by 2020.

a Focus on non communicable diseases, constituting 60% of deaths in India.

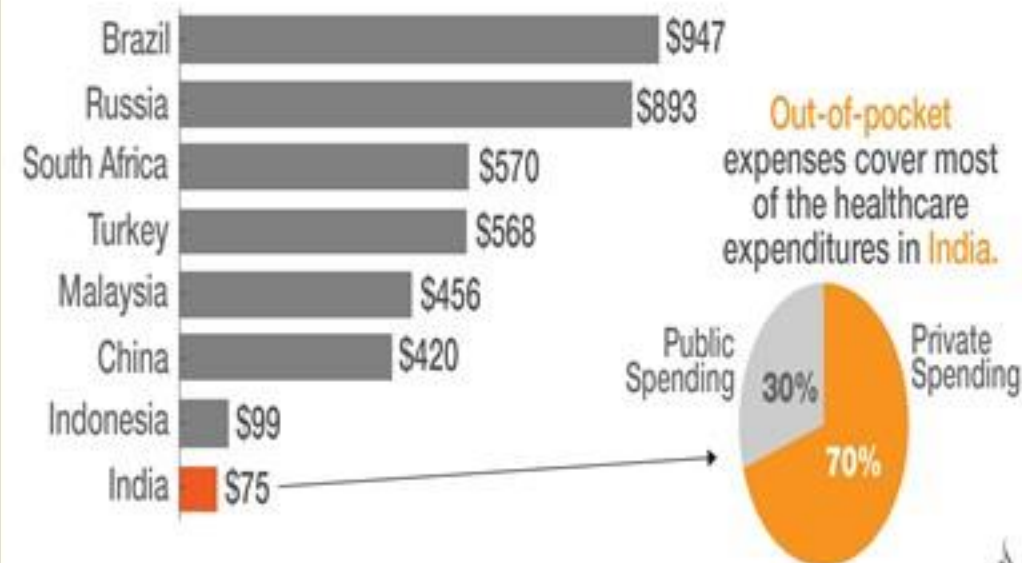
b Collaboration and regulating private sector that provides 2/3rd services.

c **Shift from sick-care to wellness:** 2/3rd budget to primary health care

d Intersectoral approach, urban health care (pollution, violence, stress etc.)

Health expenditure per person

Among the BRICS and other newly industrialised nations, India spends the least on health per capita.



Source: World Health Organization

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NATIONAL HEALTH POLICY, 2017

2 Affordable quality healthcare for all:

Universal access to drugs and diagnostics, secondary and tertiary services

3 Mainstream AYUSH**4 Targets:** increase life expectancy to 70 years from 67.5 years by 2025**5 Negatives:**

1. Issues like Right to Health, increasing public spending by 2020 and imposing health cess are ignored.
2. Doesn't speak about public health education.



National Health Policy 2017

Preventive and promotive focus with pluralistic choice



Creation of Public Health Management Cadre in all States to optimise health outcomes.

Interventions from early detection of issues in childhood to prevention of chronic illnesses.

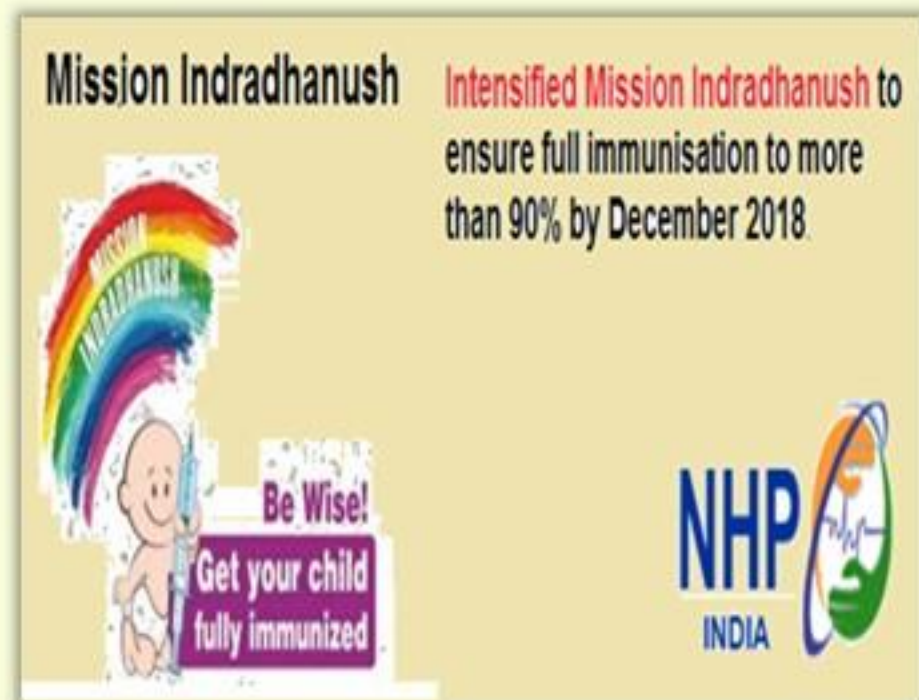


Tracking behaviour change, education and counselling at all levels.

Plethora of options to choose from among yoga and AYUSH umbrella of remedies



- 1 PMO advanced deadline from 2020 to 2018
- 2 To be regulated through PRAGATI and using eVIN technology system
- 3 Under Universal Immunisation Programme 1985 - currently 7 free vaccines for children under 2 and pregnant women



- 1 Recently NITI Aayog proposed partial privatisation of district-level government hospitals through 30 year lease bids.
- 2 **Need to follow:** WHO's Health in All Policies (HiAP) framework was established wherein health considerations are made in policymaking across different sectors, such as power, transport, agriculture and housing, that could influence health



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